Confirm your alectinib treatment setting This guide covers alectinib alone for ALK-positive lung cancer. Confirm whether it is adjuvant treatment after complete surgery, first treatment for advanced cancer, or treatment after crizotinib. These settings have different evidence and treatment lengths.

NATIONAL CANCER CONTROL PROGRAMME (NCCP) 00401 · VERSION 6

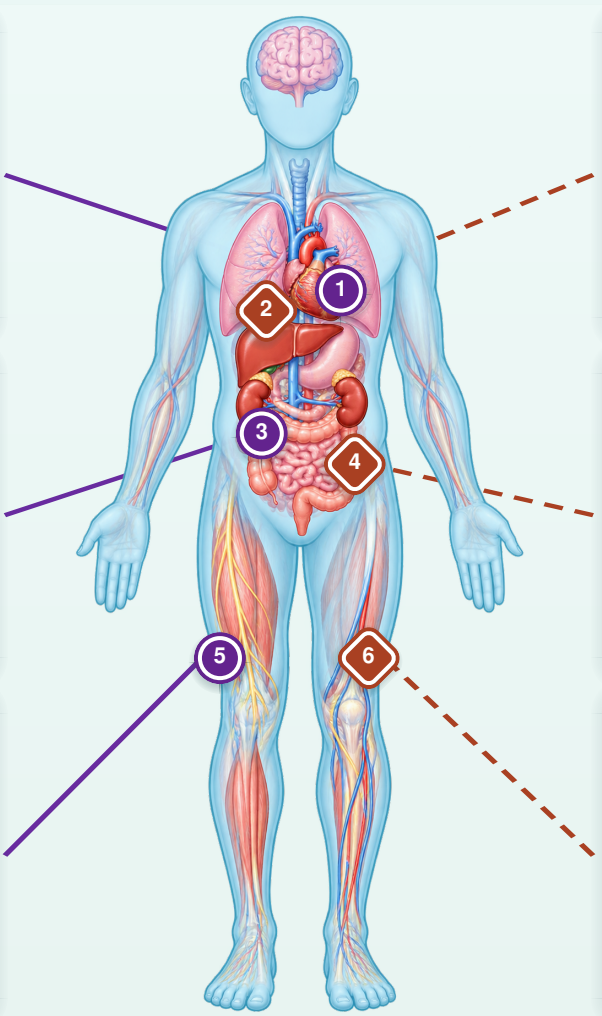
Alectinib monotherapy

Take alectinib twice daily with food, every day, with no planned rest week. After surgery, treatment lasts up to 24 months, stopping sooner for recurrence or unacceptable side effects. For advanced cancer, treatment continues until progression or unacceptable side effects under the oncology plan.

ALK-targeted oral treatment · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 New cough or breathlessness

alectinib

Report promptly because lung inflammation can occur.

2 Dizziness or fainting

alectinib

A slow pulse can be symptomatic and needs medicine review.

3 Liver tests / jaundice

alectinib

Liver injury may start silently; keep tests and report yellow skin or dark urine.

4 Fatigue or pallor

alectinib

Anaemia can include red-cell breakdown; report marked changes.

5 Muscle pain or weakness

alectinib

creatine phosphokinase (CPK) tests help assess muscle injury; report unexplained symptoms.

6 Sun sensitivity

alectinib

Use high-factor sun protection during treatment and for 7 days after stopping.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team promptly for new or worsening cough or breathlessness, unexplained muscle pain or weakness, yellow skin or eyes, dark urine, marked tiredness or pallor, dizziness or fainting, severe abdominal pain, persistent vomiting or diarrhoea, or reduced urine. Sudden serious breathing difficulty, severe chest pain or collapse needs emergency help. Do not wait for the next blood test. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Take alectinib twice daily with food, every day, with no planned rest week. After surgery, treatment lasts up to 24 months, stopping sooner for recurrence or unacceptable side effects. For advanced cancer, treatment continues until progression or unacceptable side effects under the oncology plan.

The usual starting dose is 600 mg twice daily: four 150 mg capsules each time, taken with food. Use your own prescription, which may specify a lower dose. Swallow capsules whole; do not open or dissolve them. If a dose is missed, take it only if the next dose is more than 6 hours away. If the next dose is due within 6 hours, skip the missed dose. Do not take two doses together. If you vomit after a dose, take the next dose at its usual time; do not take an extra dose.

Checks and follow-up

Liver profile every 2 weeks for the first 3 months, then every 4 weeks or as clinically indicated. full blood count (FBC) and renal profile every 4 weeks. CPK every 2 weeks for the first month, then as clinically indicated, including muscle symptoms. Blood pressure and pulse as clinically indicated. NCCP says electrocardiogram (ECG) prior to cycle 2, then as indicated; a continuous oral regimen has no defined cycle length, so agree the first repeat ECG appointment locally. Assess symptoms, interactions, adherence, current dose and treatment benefit; scans and disease monitoring follow the Consultant plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Anti-sickness medicines

Alectinib is minimal-to-low oral emetic risk. Treat nausea with prescribed as-required medicine if needed. Routine preventive anti-sickness treatment and infusion premedication are not required; your team selects a suitable rescue medicine.

Capsules, food and missed doses

The usual starting dose is 600 mg twice daily: four 150 mg capsules each time, taken with food. Use your own prescription, which may specify a lower dose. Swallow capsules whole; do not open or dissolve them. If a dose is missed, take it only if the next dose is more than 6 hours away. If the next dose is due within 6 hours, skip the missed dose. Do not take two doses together. If you vomit after a dose, take the next dose at its usual time; do not take an extra dose.

Muscles, liver, blood and breathing

Keep liver, CPK and blood-count appointments. Muscle symptoms, jaundice, dark urine, unusual fatigue, cough or breathlessness need prompt review even between appointments.

Heart rate and other medicines

Tell the team about dizziness, fainting and medicines that slow the pulse. Check new prescriptions, supplements, grapefruit and Seville oranges with pharmacy; interactions may require closer monitoring rather than an automatic dose change.

Sun protection

Avoid prolonged sunlight during treatment and for at least 7 days after stopping. Use broad-spectrum UVA/UVB sunscreen and lip balm sun protection factor (SPF) 50 or higher.

Pregnancy, contraception and breastfeeding

Women who could become pregnant need highly effective contraception during treatment and for at least 5 weeks afterwards. Men with female partners who could become pregnant need highly effective contraception during treatment and for at least 3 months afterwards. Oral contraceptive effectiveness may be reduced: ask for a suitable method. Do not breastfeed during treatment. Discuss fertility before treatment and contact the team promptly if pregnancy occurs.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital's urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00401/

Public information only. No personal information is encoded.